

In the Supreme Court of the United States

UNITED STATES OF AMERICA, ET AL., APPLICANTS

v.

EMILY SHILLING, ET AL.

**APPLICATION FOR A STAY OF THE INJUNCTION
ISSUED BY THE UNITED STATES DISTRICT COURT
FOR THE WESTERN DISTRICT OF WASHINGTON**

D. JOHN SAUER
Solicitor General
Counsel of Record
Department of Justice
Washington, D.C. 20530-0001
SupremeCtBriefs@usdoj.gov
(202) 514-2217

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PARTIES TO THE PROCEEDING

Applicants (defendants-appellants below) are the United States of America; Donald J. Trump, President of the United States, in his official capacity; Peter Hegseth, Secretary of Defense, in his official capacity; the United States Department of Defense; Daniel P. Driscoll, Secretary of the Army, in his official capacity; the United States Department of the Army; John C. Phelan, Secretary of the Navy, in his official capacity; the United States Department of the Navy; Gary Ashworth, Acting Secretary of the Air Force, in his official capacity; and the United States Department of the Air Force.*

Respondents (plaintiffs-appellees below) are Commander Emily Shilling; Commander Blake Dremann; Lieutenant Commander Geirid Morgan; Sergeant First Class Cathrine Schmid; Sergeant First Class Jane Doe; Staff Sergeant Videll Leins; Sergeant First Class Sierra Moran; Matthew Medina; and Gender Justice League.

* After applicants filed their notice of appeal to the Ninth Circuit, the district court dismissed Donald J. Trump, in his official capacity as President of the United States, as a defendant in this case. See D. Ct. Docket Entry No. 106 (Mar. 28, 2025).

RELATED PROCEEDINGS

United States District Court (W.D. Wash.):

Shilling v. United States, No. 25-cv-241 (Mar. 27, 2025)

United States Court of Appeals (9th Cir.):

Shilling v. Trump, No. 25-2039 (Apr. 18, 2025)

In the Supreme Court of the United States

No. 24A

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v.

EMILY SHILLING, ET AL.

APPLICATION FOR A STAY OF THE INJUNCTION ISSUED BY THE UNITED STATES DISTRICT COURT FOR THE WESTERN DISTRICT OF WASHINGTON

Pursuant to Rule 23 of the Rules of this Court and the All Writs Act, 28 U.S.C. 1651, the Solicitor General—on behalf of applicants the United States, et al.—respectfully files this application for a stay of the injunction issued by the U.S. District Court for the Western District of Washington (App., *infra*, 190a-256a) pending the consideration and disposition of the government’s appeal to the United States Court of Appeals for the Ninth Circuit and pending any further review in this Court.

In this case, the district court issued a universal injunction usurping the Executive Branch’s authority to determine who may serve in the Nation’s armed forces—despite this Court previously staying injunctions against a materially indistinguishable policy. In February 2025, the Department of Defense adopted its current policy, which generally disqualifies from military service individuals who have gender dysphoria or have undergone medical interventions for gender dysphoria. The policy was based in part on the findings of a panel of experts convened during the first Trump Administration, which found that service by individuals with gender dys-

phoria was contrary to “military effectiveness and lethality.” App., *infra*, 8a. Respondents challenged the Department’s 2025 policy on equal protection and other grounds. Despite this Court’s admonition that professional military judgments about the composition of the armed forces should be given “great deference,” *Winter v. Natural Res. Def. Council, Inc.*, 555 U.S. 7, 24 (2008) (citation omitted), the district court issued a universal injunction against the 2025 policy. In so doing, the court ordered the government to maintain a different policy that the Department has deemed “incompatible with the high mental and physical standards necessary for military service.” App., *infra*, 124a.

As noted, this case does not plow new ground. In 2018, then-Secretary of Defense James Mattis adopted a policy, materially indistinguishable from the one at issue here, that generally disqualified individuals with gender dysphoria from military service. Certain plaintiffs challenged the Mattis policy on equal protection and other grounds, and district courts enjoined the policy on a universal basis.

This Court stayed those injunctions, see *Trump v. Karnoski*, 586 U.S. 1124 (2019); *Trump v. Stockman*, 586 U.S. 1124 (2019), and it should do the same here. Because the 2025 policy, like the Mattis policy, turns on a medical condition (gender dysphoria) and related medical interventions, the Constitution demands only rational-basis review, especially in the military context. The 2025 policy, like the Mattis policy, readily satisfies that review. It is *undisputed* that gender dysphoria is a medical condition associated with clinically significant distress or impairment in social, occupational, or other important areas of functioning. App., *infra*, 185a. And in the 2025 policy, as in the Mattis policy before it, the Department rationally determined that service by individuals with gender dysphoria would undermine military effectiveness and lethality—consistent with similar, longstanding determinations for

a wide range of other medical conditions (such as asthma and hypertension). *Id.* at 74a, 93a; D. Ct. Doc. 73-5, at 15 (Mar. 13, 2025).

The district court's injunction cannot be squared with the substantial deference that the Department's professional military judgments are owed. Nor can the court's injunction be squared with this Court's decisions to stay the injunctions against the Mattis policy. In staying those injunctions, this Court necessarily determined that the government was likely to succeed in defending the Mattis policy on the merits. Yet the district court failed to identify any relevant difference in the 2025 policy that would justify a different conclusion here. Although the court noted that the 2025 policy narrows the circumstances in which an otherwise disqualified person may nonetheless serve, App., *infra*, 193a, the 2025 policy merely reflects a more cautious approach to the risks associated with gender dysphoria and related medical interventions, *id.* at 122a. Nothing in the Constitution prohibits the Department from making that modest change, which does not alter the conclusion that rational-basis review applies and is satisfied. And in suggesting that experience under the policies of the last Administration shows that service by individuals with gender dysphoria does not present meaningful risks, *id.* at 235a-236a, the court improperly second-guessed the military's own assessment of those risks, *id.* at 122a.

The Ninth Circuit's denial of a stay likewise cannot be reconciled with this Court's decisions to stay the injunctions against the Mattis policy. In a one-page order, the Ninth Circuit asserted that the government had not shown that it would suffer irreparable harm absent a stay. But in staying the injunctions against the Mattis policy, this Court necessarily determined that those injunctions would otherwise cause the government irreparable harm. The same is true here: Like the injunctions against the Mattis policy, the injunction against the 2025 policy irreparably

harms the government by forcing it to maintain a policy that the Department has found to be inconsistent with “the best interests of the Military Services” and with “the interests of national security.” App., *infra*, 126a.

Absent a stay, the district court’s universal injunction will remain in place for the duration of further review in the Ninth Circuit and in this Court—a period far too long for the military to be forced to maintain a policy that it has determined, in its professional judgment, to be contrary to military readiness and the Nation’s interests. The government therefore respectfully requests a stay of the injunction in its entirety pending further review. At minimum, this Court should stay the injunction’s universal scope, so that the injunction blocks the implementation of the 2025 policy only as to the eight individual respondents in this case.

STATEMENT

A. The Military’s Policies

1. To assemble a military of “qualified, effective, and able-bodied persons,” 10 U.S.C. 505(a), the Department of Defense (Department or DoD) has traditionally set demanding standards for military service, App., *infra*, 12a. The Department’s accession standards—the standards that govern entry into the military—list hundreds of medical conditions that are disqualifying absent a waiver. *Id.* at 67a-108a. The list includes cataracts, asthma, high blood pressure (hypertension), diabetes, severe headaches, and other common medical conditions. *Id.* at 69a, 74a, 93a, 98a, 102a. The list also contains various mental, behavioral, and learning disorders, including autism spectrum disorders, bipolar and related disorders, eating disorders, suicidality, obsessive-compulsive disorders, anxiety and depressive disorders in specified circumstances, and substance-related and addictive disorders (except using caffeine or tobacco). *Id.* at 104a-106a. Because of the Department’s rigorous accession

standards, “[t]he vast majority of Americans from ages 17 to 24—that is, 71%—are ineligible to join the military without a waiver for mental, medical, or behavioral reasons.” *Id.* at 16a.

In general, the Department has aligned the mental disorders it has deemed presumptively disqualifying with those listed in the *Diagnostic and Statistical Manual of Mental Disorders (DSM)*, published by the American Psychiatric Association (APA). App., *infra*, 20a. The 1980 edition of the *DSM* recognized “[t]ranssexualism” as a disorder whose “essential features” were “a persistent sense of discomfort and inappropriateness about one’s anatomic sex and a persistent wish to be rid of one’s genitals and to live as a member of the other sex.” APA, *Diagnostic and Statistical Manual of Mental Disorders* 261-262 (3d ed. 1980) (*DSM-III*). When the *DSM* was updated in 1994, “[t]ranssexualism” was subsumed within the term “[g]ender [i]dentity [d]isorder.” APA, *Diagnostic and Statistical Manual of Mental Disorders* 785 (4th ed. 1994) (*DSM-IV*). Consistent with the *DSM*, the military’s accession standards had for decades treated a “history of * * * transsexualism” as disqualifying. App., *infra*, 20a (brackets and citation omitted). Likewise, the military’s retention standards—the standards that govern the retention and separation of persons already serving in the military—had historically treated transsexualism as a “permissible basis” for discharging a servicemember. *Id.* at 17a. Again, that was consistent with the Department’s demanding standards for military service, which identify mental disorders recognized by the *DSM* as permissible grounds for discharging servicemembers. See, e.g., D. Ct. Doc. 73-5, at 36-37.

2. In 2013, when the APA published the fifth edition of the *DSM* (known as the *DSM-5*), it replaced the term “gender identity disorder” with “gender dysphoria.” App., *infra*, 22a. The *DSM-5* defines gender dysphoria as a “marked incongru-

ence between one's experienced/expressed gender and assigned gender, of at least 6 months' duration," that is "manifested" in various specified ways and is "associated with clinically significant distress or impairment in social, occupational, or other important areas of functioning." APA, *Diagnostic and Statistical Manual of Mental Disorders* 452-453 (5th ed. 2013) (*DSM-5*); see App., *infra*, 185a. According to the *DSM-5*, some adults with gender dysphoria may desire cross-sex hormone interventions or sex-reassignment surgery. *DSM-5*, at 454; see App., *infra*, 32a. The *DSM-5* further observes that adults with gender dysphoria are at "increased risk for suicidal ideation, suicide attempts, and suicides," *DSM-5*, at 454, and "may have coexisting mental health problems, most commonly anxiety and depressive disorders," *id.* at 459.

Under the *DSM-5*, "[g]ender dysphoria" and "[t]ransgender" are distinct terms: Whereas "[t]ransgender" refers to the broad spectrum of individuals who transiently or persistently identify with a gender different from their natal [*i.e.*, birth] gender," "[g]ender dysphoria" refers to the distress that may accompany the incongruence between one's experienced or expressed gender and one's assigned gender." *DSM-5*, at 451. The *DSM-5* emphasizes that "not all individuals will experience distress as a result of such incongruence" and that the term "[g]ender dysphoria," unlike the previous term "*gender identity disorder*," "focuses on dysphoria as the clinical problem, not identity per se." *Ibid.*; see App., *infra*, 30a ("'[N]ot all transgender people suffer from gender dysphoria and that distinction,' according to the APA, 'is important to keep in mind.'") (quoting APA, *Expert Q & A: Gender Dysphoria*).

3. In 2016, then-Secretary of Defense Ashton Carter ordered the military to adopt new accession standards by July 2017. App., *infra*, 24a. Under the new accession standards, a history of transsexualism would no longer be disqualifying.

But Secretary Carter recognized the need for “[m]edical standards” that “ensure that those entering service are free of medical conditions or physical defects that may require excessive time lost from duty.” *Id.* at 4a. Secretary Carter therefore ordered that the new accession standards treat a “history of gender dysphoria,” a “history of medical treatment associated with gender transition,” and a “history of sex reassignment or genital reconstruction surgery” as “disqualifying,” with certain exceptions. *Id.* at 25a-26a. For example, Secretary Carter ordered that the new accession standards treat a “history of gender dysphoria” as “disqualifying, unless, as certified by a licensed medical provider, the applicant has been stable without clinically significant distress or impairment in social, occupational, or other important areas of functioning for 18 months.” *Id.* at 25a.

Secretary Carter also ordered the military to adopt new retention standards, effective immediately. App., *infra*, 24a. Under the new retention standards, servicemembers who were diagnosed with gender dysphoria were permitted to undergo “gender transition” and serve in their “transitioned gender.” *Id.* at 24a-25a. But servicemembers who did “not meet the clinical criteria for gender dysphoria” “remain[ed] subject to the standards and requirements applicable to their biological sex.” *Id.* at 25a. Thus, absent a diagnosis of the clinically significant distress or impairment that constitutes gender dysphoria, servicemembers had to use the berthing, bathroom, and shower facilities corresponding to their sex even if they personally identified with the opposite sex. See *id.* at 39a. They likewise had to meet the physical-fitness, body-fat, uniform, and grooming standards applicable to their sex. See *id.* at 39a-40a.

4. In June 2017—before the Carter accession standards were set to take effect—then-Secretary of Defense James Mattis determined, “after consultation with the Secretaries and Chiefs of Staff of each Service,” that it was necessary to defer

those standards until the military could “evaluate more carefully” their potential effect “on readiness and lethality.” App., *infra*, 27a (citation omitted). Secretary Mattis established a panel of experts to “conduct an independent multi-disciplinary review and study of relevant data and information pertaining to transgender Service members.” *Ibid.* (citation omitted). The panel consisted of “senior uniformed and civilian Defense Department and U.S. Coast Guard leaders.” *Id.* at 7a. After “extensive review and deliberation” over several months, the panel “exercised its professional military judgment” and presented its independent recommendations to the Secretary. *Id.* at 28a.

In February 2018, Secretary Mattis sent President Trump a memorandum proposing a new policy consistent with the panel’s conclusions, along with a lengthy report explaining the policy. App., *infra*, 7a-54a. Recognizing that the new policy reflected “the exercise of [Secretary Mattis’s] independent judgment,” the President issued a memorandum permitting the Secretaries of Defense and Homeland Security “to implement” it. *Military Service by Transgender Individuals*, 83 Fed. Reg. 13,367, 13,367 (Mar. 23, 2018).

Under the Mattis policy, individuals “who [we]re diagnosed with, or ha[d] a history of, gender dysphoria [we]re generally disqualified from accession or retention in the Armed Forces,” except under “limited circumstances.” App., *infra*, 52a. The Mattis accession standards thus treated a “history of gender dysphoria” as disqualifying, unless applicants could “demonstrate 36 consecutive months of stability—i.e., absence of gender dysphoria—immediately preceding their application; they ha[d] not transitioned to the opposite gender; and they [we]re willing and able to adhere to all standards associated with their biological sex.” *Ibid.* Similarly, the Mattis retention standards generally treated a diagnosis of gender dysphoria as cause for discharge,

unless a servicemember was “willing and able to adhere to all standards associated with [the servicemember’s] biological sex, the Service member d[id] not require gender transition, and the Service member [wa]s not otherwise non-deployable for more than 12 months or for a period of time in excess of that established by Service policy (which may be less than 12 months).” *Ibid.* In addition, the Mattis policy, like the Carter policy before it, required servicemembers without a history or diagnosis of gender dysphoria to serve in their “biological sex.” *Id.* at 9a.

Several district courts entered universal preliminary injunctions against the Mattis policy on the view that the policy was a “ban on military service by openly transgender people.” *Karnoski v. Trump*, No. 17-cv-1297, 2018 WL 1784464, at *1 (W.D. Wash. Apr. 13, 2018), vacated, 926 F.3d 1180 (9th Cir. 2019); see *Doe v. Trump*, 315 F. Supp. 3d 474, 492 (D.D.C. 2018), rev’d, 755 Fed. Appx. 19 (D.C. Cir. 2019); *Stockman v. Trump*, 331 F. Supp. 3d 990, 1001 (C.D. Cal. 2018), vacated, No. 18-56539, 2019 WL 6125075 (9th Cir. Aug. 26, 2019). In each case, the government sought from this Court a grant of certiorari before judgment or, in the alternative, a stay of the injunction pending appeal. See *Trump v. Karnoski*, Nos. 18-676, 18A625; *Trump v. Doe*, Nos. 18-677, 18A626; *Trump v. Stockman*, Nos. 18-678, 18A627. The government emphasized that “the Mattis policy turn[ed] on a medical condition (gender dysphoria) and related treatment (gender transition)” —rather than transgender status—and was consistent with equal protection, due process, and the First Amendment. Pet. at 19, *Trump v. Karnoski*, No. 18-676 (Nov. 23, 2018); see *id.* at 19-25. While the government’s requests were pending in this Court, the D.C. Circuit vacated the injunction in one of the cases. See *Doe v. Shanahan*, 917 F.3d 694 (2019) (separate opinions); *Doe v. Shanahan*, 755 Fed. Appx. 19 (2019) (per curiam). Soon after,

this Court granted stays of the injunctions in the other two cases. See *Trump v. Karnoski*, 586 U.S. 1124 (2019); *Trump v. Stockman*, 586 U.S. 1124 (2019).

5. In 2021, President Biden revoked President Trump’s memorandum permitting implementation of the Mattis policy. Exec. Order No. 14,004, § 2 (Jan. 25, 2021), 86 Fed. Reg. 7471, 7472 (Jan. 28, 2021). Thereafter, then-Secretary of Defense Lloyd Austin issued new accession standards, but those standards still treated a “[h]istory of gender dysphoria,” a history of “gender affirming surgery,” and a “[h]istory of gender-affirming hormone therapy” as disqualifying under specified circumstances. App., *infra*, 82a, 84a, 100a, 106a. For example, the Austin accession standards treated a “[h]istory of gender dysphoria” as disqualifying if the condition was “[s]ymptomatic within the previous 18 months” or was “[a]ssociated with comorbid mental health disorders.” *Id.* at 106a. Secretary Austin also issued new retention standards that permitted servicemembers diagnosed with gender dysphoria to undergo “gender transition” and serve in their asserted gender identity, but that required servicemembers without such a diagnosis to serve in their sex. DoD Instruction 1300.28, *In-Service Transition for Transgender Service Members* 7-8 (Apr. 30, 2021).

6. Soon after taking office in January 2025, President Trump issued Executive Order No. 14,183, declaring it to be “the policy of the United States Government to establish high standards for troop readiness, lethality, cohesion, honesty, humility, uniformity, and integrity.” App., *infra*, 114a. Finding that policy to be “inconsistent with the medical, surgical, and mental health constraints on individuals with gender dysphoria,” *ibid.*, the President ordered the Department to update the military’s accession and retention standards, *id.* at 114a-115a.

On February 26, 2025, the Department issued a new policy with updated accession and retention standards. App., *infra*, 124a-136a. The 2025 policy was based on “consideration of, among other things, the President and Secretary’s written direction, existing and prior DoD policy, and prior DoD studies and reviews of service by individuals with gender dysphoria.” *Id.* at 121a. That consideration included Secretary Mattis’s 2018 memorandum, which determined, based on the work of a panel of experts, that “there are substantial risks associated with allowing accession and retention of individuals with a history or diagnosis of gender dysphoria”; a 2021 review conducted by the Department’s Psychological Health Center of Excellence and the Accession Medical Standards Analysis and Research Activity, which “found that nearly 40% of Service members with gender dysphoria in an observed cohort were non-deployable over a 24 month period”; a 2025 medical-literature review conducted by the Office of the Assistant Secretary of Defense for Health Affairs, which reported that “the suicide attempt rate is estimated to be 13 times higher among transgender individuals compared to their cisgender counterparts,” and that “the strength of evidence on transgender mental health and gender-affirming care is low to moderate”; and a review of cost data by the Office of the Assistant Secretary of Defense for Health Affairs, which found that, “between 2015 and 2024, DoD spent \$52,084,407 providing care to active duty Service members to treat gender dysphoria.” *Id.* at 121a-122a. Based on its review, the Department determined that, “[w]hile Service members with gender dysphoria volunteered to serve their country, the costs associated with their health care, coupled with the medical and readiness risks associated with their diagnosis and associated treatment that can limit their deployability, make continued service by such individuals incompatible with the Department’s rigorous standards and national security imperative to deliver a ready, deployable force.” *Id.* at 122a.

Under the 2025 policy, as under the Mattis policy, individuals “who have a current diagnosis or history of, or exhibit symptoms consistent with, gender dysphoria” are presumptively disqualified from accession and retention. App., *infra*, 129a, 131a. “[E]xhibit[ing] symptoms consistent with gender dysphoria” means “exhibit[ing] such symptoms as would be sufficient to constitute a diagnosis” under the criteria set forth in the *DSM-5*. *Id.* at 181a n.2. The 2025 policy likewise treats a “history of cross-sex hormone therapy or sex reassignment or genital reconstruction surgery as treatment for gender dysphoria or in pursuit of a sex transition” as disqualifying for accession and retention. *Id.* at 129a, 131a. Under the 2025 policy, however, an individual with a disqualifying history or condition “may be considered for a retention or accession waiver on a case-by-case basis where there is a compelling Government interest * * * that directly supports the Department’s warfighting capabilities,” such as “special experience, special training, and advanced education in a highly technical career field designated as mission critical and hard to fill.” *Id.* at 179a. To be eligible for such a waiver, an individual must meet criteria similar to the criteria set forth in the Mattis policy: The individual must “demonstrate[] 36 consecutive months of stability in the individual’s sex”; the individual must “demonstrate[] that he or she has never attempted to transition to any sex other than his or her sex”; and the individual must be “willing and able to adhere to all applicable standards, including the standards associated with his or her sex.” *Id.* at 179a-180a. Finally, the 2025 policy, like the Carter, Mattis, and Austin policies before it, requires servicemembers without gender dysphoria to serve “in accordance with their sex.” *Id.* at 126a-127a.

B. Procedural History

Respondents are seven trans-identifying individuals who are currently serving in the military, one trans-identifying individual who wishes to join the military, and

an advocacy organization whose members include three of the individual respondents. See App., *infra*, 118a, 140a-141a. Characterizing Executive Order No. 14,183 and the 2025 policy as a “ban” on military service by “transgender people,” *id.* at 138a, respondents sued in the Western District of Washington, challenging the “[b]an” as a violation of equal protection, the First Amendment, procedural due process, and the doctrine of equitable estoppel, *id.* at 168a-175a. Respondents moved for a universal preliminary injunction. D. Ct. Doc. 23 (Feb. 19, 2025).

On March 27, 2025, the district court granted respondents’ motion and entered a universal preliminary injunction prohibiting the government from implementing Executive Order No. 14,183 and the 2025 policy and requiring the government to maintain the Austin policy. App., *infra*, 190a-191a. Agreeing with respondents’ characterization of the 2025 policy as “a *de facto* blanket prohibition on transgender service,” *id.* at 193a, the court held that respondents were likely to succeed on their equal protection claim, *id.* at 217a-237a. The court concluded that the 2025 policy was subject to intermediate scrutiny because the policy discriminated based on transgender status and sex. *Id.* at 220a-226a. The court further concluded that the 2025 policy failed intermediate scrutiny because “banning transgender persons from serving” was not “substantially related to achieving military readiness,” *id.* at 231a; to “achieving unit cohesion, good order, or discipline,” *id.* at 234a; or to “cost effectiveness,” *id.* at 235a. The court also held that the 2025 policy would not survive even rational-basis review because the 2025 policy “reli[ed] on seven-year-old predictions from the Mattis Policy” without citing “updated data.” *Id.* at 235a-236a. The court likewise held that respondents were likely to succeed on their First Amendment, procedural due process, and equitable estoppel claims. *Id.* at 237a-249a. And it concluded that

respondents had satisfied the remaining preliminary injunction factors. *Id.* at 249a-253a. The court denied the government’s request for a stay. *Id.* at 188a-189a.

The government appealed and asked the Ninth Circuit for an administrative stay and a stay pending appeal. On March 31, the Ninth Circuit denied the government’s request for an administrative stay. App., *infra*, 257a. Nearly three weeks later, on April 18, the Ninth Circuit issued a one-page order denying the government’s request for a stay pending appeal. *Id.* at 258a. The court stated that the government had not shown that it “will suffer irreparable harm absent a stay.” *Ibid.**

ARGUMENT

Under Rule 23 of the Rules of this Court and the All Writs Act, 28 U.S.C. 1651, the Court may stay a district order’s interlocutory order granting emergency relief. See, e.g., *Trump v. International Refugee Assistance Project*, 582 U.S. 571 (2017) (per curiam); *Brewer v. Landrigan*, 562 U.S. 996 (2010); *Brunner v. Ohio Republican Party*, 555 U.S. 5, 6 (2008). An applicant must show (1) a likelihood of success on the merits, (2) a reasonable probability of obtaining certiorari, and (3) a likelihood of irreparable harm. See *Hollingsworth v. Perry*, 558 U.S. 183, 190 (2010) (per curiam). In “close cases,” “the Court will balance the equities and weigh the relative harms.” *Ibid.*

* Two other challenges to the 2025 policy are pending in other courts. In *Talbott v. Trump*, No. 25-cv-240 (D.D.C.), the district court issued a universal preliminary injunction barring the government from implementing Executive Order No. 14,183 and the 2025 policy, 2025 WL 842332, at *3 (Mar. 18, 2025), and denied the government’s motion to dissolve the injunction, 2025 WL 914716, at *2 (Mar. 26, 2025). The government appealed and sought an administrative stay and a stay pending appeal. The D.C. Circuit granted an administrative stay and heard oral argument on the government’s motion for a stay pending appeal on April 22. See *Talbott v. United States*, No. 25-5087 (2025). In *Ireland v. Hegseth*, No. 25-cv-1918 (D.N.J.), the district court issued a 14-day temporary restraining order (TRO) barring the government from implementing Executive Order No. 14,183 and the 2025 policy as to the two named plaintiffs in that case. See D. Ct. Doc. 28, at 8 (Mar. 24, 2025), *Ireland*, *supra* (No. 25-cv-1918). That TRO has expired, and the court has not renewed it.

All of those factors strongly support a stay of the district court’s universal injunction against the 2025 policy in this case. Indeed, this Court has previously stayed universal injunctions blocking the Mattis policy—a DoD policy that likewise presumptively disqualified individuals with gender dysphoria from military service. See *Trump v. Karnoski*, 586 U.S. 1124 (2019); *Trump v. Stockman*, 586 U.S. 1124 (2019). There is no basis for a different outcome here.

I. THE GOVERNMENT IS LIKELY TO SUCCEED ON THE MERITS

In granting stays of the injunctions against the Mattis policy, this Court necessarily determined that the government was likely to succeed on the merits of the plaintiffs’ challenges to that policy. See *Hollingsworth*, 558 U.S. at 190 (“To obtain a stay * * *, an applicant must show * * * a fair prospect that a majority of the Court will vote to reverse the judgment below.”). Respondents raise similar challenges to the 2025 policy, and neither court below identified any relevant difference between that policy and the Mattis policy. Accordingly, this Court should reach the same conclusion as it did before: The government is likely to succeed on the merits.

A. The 2025 Policy Is Consistent With Equal Protection

The 2025 policy does not discriminate against any suspect or quasi-suspect class. Instead, like the Carter, Mattis, and Austin policies before it, the 2025 policy draws classifications based on a medical condition (gender dysphoria) and related medical interventions. Thus, especially given the military context, the 2025 policy warrants only rational-basis review, which it easily satisfies.

1. The 2025 policy warrants only rational-basis review

a. Gender dysphoria is a medical condition “associated with clinically significant distress or impairment in social, occupational, or other important areas of functioning.” *DSM-5*, at 453. The 2025 policy presumptively disqualifies from mili-

tary service individuals with gender dysphoria. App., *infra*, 129a, 131a. It also disqualifies from military service individuals who have received interventions related to gender dysphoria—*i.e.*, “cross-sex hormone therapy or sex reassignment or genital reconstruction surgery.” *Id.* at 129a; see *id.* at 131a.

The 2025 policy thus turns on a medical condition (gender dysphoria) and related medical interventions. There is nothing suspect about those classifications. The same classifications appear in the *DSM-5*, which identifies “gender dysphoria” as a medical condition that may lead some to seek “hormone treatment” or “gender reassignment surgery.” *DSM-5*, at 454. The same classifications also appear in the Department’s prior Carter, Mattis, and Austin policies. See pp. 6-10, *supra*. For example, the Carter accession standards treated a history of “gender dysphoria,” “medical treatment associated with gender transition,” or “sex reassignment or genital reconstruction surgery” as “disqualifying,” with certain exceptions. App., *infra*, 25a-26a. Likewise, the Austin accession standards—which the district court’s injunction requires the military to keep in place—treats a history of “gender dysphoria,” “gender affirming surgery,” or “gender-affirming hormone therapy” as disqualifying under specified circumstances. *Id.* at 82a, 84a, 100a, 106a.

Because the 2025 policy, like the Carter, Mattis, and Austin policies before it, draws classifications based on a medical condition and related medical interventions, the 2025 policy warrants only rational-basis review. See, *e.g.*, *Board of Trs. of the Univ. of Ala. v. Garrett*, 531 U.S. 356, 365-368 (2001). Because that would be true even in the civilian context, it follows *a fortiori* in the military context. Any greater level of scrutiny would conflict with the “healthy deference” that the political branches are owed “in the area of military affairs.” *Rostker v. Goldberg*, 453 U.S. 57, 66 (1981) (upholding Congress’s decision to exclude women from Selective Service

registration). That deference reflects the recognition “[n]ot only” that “courts [are] ill-equipped to determine the impact upon discipline that any particular intrusion upon military authority might have,” but also that “military authorities have been charged by the Executive and Legislative Branches with carrying out our Nation’s military policy.” *Goldman v. Weinberger*, 475 U.S. 503, 507-508 (1986) (upholding Air Force’s decision to prohibit clinical psychologist from wearing a yarmulke while in uniform) (citation omitted); see *Winter v. Natural Res. Def. Council, Inc.*, 555 U.S. 7, 24 (2008) (explaining that “complex, subtle, and professional decisions as to the composition * * * of a military force” are “essentially professional military judgments”) (citation omitted). Thus, even if an analogous policy in the civilian context would somehow call for closer scrutiny despite being based on a medical condition rather than transgender status, the military’s 2025 policy would still warrant the most deferential standard of review. See *Rostker*, 453 U.S. at 67 (“[T]he tests and limitations to be applied may differ because of the military context.”); cf. *Goldman*, 475 U.S. at 507 (explaining that judicial “review of military regulations challenged on First Amendment grounds is far more deferential than constitutional review of similar laws or regulations designed for civilian society”).

b. The district court nevertheless concluded that the 2025 policy triggers intermediate scrutiny. App., *infra*, 220a-226a. But none of its rationales is tenable, especially since all of them would equally condemn the Austin policy that the court ordered the Department to maintain.

The district court primarily sought to justify intermediate scrutiny on the ground that the 2025 policy discriminates based on transgender status. App., *infra*, 220a-223a. But the 2025 policy “focuses on dysphoria as the clinical problem, not identity per se.” *DSM-5*, at 451. Like the *DSM-5* and the Carter, Mattis, and Austin

policies, the 2025 policy uses the term gender dysphoria to “refer[] to the *distress* that may accompany the incongruence between one’s experienced or expressed gender and one’s assigned gender.” *Ibid.* (emphasis added); see App., *infra*, 181a n.2 (adopting the *DSM-5*’s “diagnostic criteria” for gender dysphoria). As the APA has emphasized, “not all individuals will experience distress as a result of such incongruence,” *DSM-5*, at 451, so “[n]ot all transgender people suffer from gender dysphoria,” App., *infra*, 30a (brackets in original) (quoting APA, *Expert Q & A: Gender Dysphoria*). “Conversely, not all persons with gender dysphoria are transgender.” *Id.* at 30a n.57; see *ibid.* (giving, as an example, men who suffer genital wounds in combat and who “feel that they are no longer men because their bodies do not conform to their concept of manliness”) (citation omitted).

The district court stated that gender dysphoria is “closely correlated” with being transgender. App., *infra*, 221a. But such correlation does not justify equating one with the other. See *Geduldig v. Aiello*, 417 U.S. 484, 496 n.20 (1974) (“While it is true that only women can become pregnant it does not follow that every legislative classification concerning pregnancy is a sex-based classification.”); see also *Dobbs v. Jackson Women’s Health Org.*, 597 U.S. 215, 236 (2022) (reaffirming the same). In any event, even if the 2025 policy discriminated based on transgender status, heightened scrutiny would still be inappropriate because trans-identifying people are not a suspect or quasi-suspect class. See *City of Cleburne v. Cleburne Living Ctr., Inc.*, 473 U.S. 432, 441-447 (1985) (declining to recognize a new quasi-suspect class).

The district court also sought to justify intermediate scrutiny on the ground that the 2025 policy discriminates based on sex. App., *infra*, 223a-226a. But the 2025 policy references “sex” in only two contexts, neither of which constitutes sex discrimination. First, certain provisions of the 2025 policy, like provisions of previous DoD

policies, reference “sex” in the context of describing certain medical interventions. See, *e.g.*, *id.* at 129a. Those provisions, however, turn on the nature of the medical intervention (*e.g.*, “sex reassignment” surgery “in pursuit of a sex transition”), not on the individual’s sex. *Ibid.* They thus do not warrant intermediate scrutiny. Second, the 2025 policy requires servicemembers who have never been diagnosed with gender dysphoria—and who have never received related interventions—to serve in their sex and to meet the standards and requirements applicable to their sex. *Id.* at 126a. But the Carter, Mattis, and Austin policies required the same thing. See pp. 7, 9, 10, *supra*. What respondents challenge is not the sex-based standards themselves—which the court’s injunction leaves in place—but the Department’s 2025 decision not to provide a *special exemption* from those sex-based standards for servicemembers who have gender dysphoria and seek to undergo gender transition and serve in their asserted gender identity. App., *infra*, 126a-127a. Failing to provide a preferential exemption from valid sex-based standards for individuals who have a particular medical condition (gender dysphoria) and who seek related interventions (gender transition) is not even arguably sex discrimination warranting intermediate scrutiny. In any event, even if the 2025 policy discriminated based on sex, intermediate scrutiny would still be inappropriate because of the deference owed to the military in this context. See *Trump v. Hawaii*, 585 U.S. 667, 703-704 (2018); *Rostker*, 453 U.S. at 66; pp. 16-17, *supra*.

2. The 2025 policy satisfies rational-basis review

The 2025 policy satisfies the deferential standard that applies here. The government has undisputedly important interests in maintaining military readiness, cohesion, good order, and discipline, as well as in managing the military’s costs. App.,

infra, 228a. And like the Mattis policy before it, the 2025 policy is rationally related to achieving those ends.

a. The Department’s accession standards have historically treated a long list of medical conditions, including mental disorders in the *DSM*, as presumptively disqualifying. App., *infra*, 67a-108a. After the APA revised the *DSM* in 2013 to identify gender dysphoria as a disorder “associated with clinically significant distress or impairment in social, occupational, or other important areas of functioning,” *DSM-5*, at 453, Secretary Carter added “gender dysphoria” to the list, along with “medical treatment associated with gender transition,” including “sex reassignment or genital reconstruction surgery,” App., *infra*, 25a-26a. Secretary Carter did so to “ensure that those entering service are free of medical conditions or physical defects that may require excessive time lost from duty.” *Id.* at 4a.

The 2025 accession standards, which likewise treat gender dysphoria and related medical interventions as presumptively disqualifying, rationally serve the same purpose. App., *infra*, 129a, 131a. To be sure, the 2025 accession standards narrow the circumstances in which an applicant may overcome the presumptive disqualification. Under the Carter accession standards, for example, a “history of gender dysphoria” is “disqualifying, unless, as certified by a licensed medical provider, the applicant has been stable without clinically significant distress or impairment in social, occupational, or other important areas of functioning for 18 months.” *Id.* at 25a. Under the 2025 accession standards, in contrast, a history of gender dysphoria is disqualifying unless an applicant obtains a waiver, which may be granted only if “there is a compelling Government interest” in allowing the applicant to join the military, the applicant “demonstrates 36 consecutive months of stability in [his or her] sex,” the applicant “demonstrates that he or she has never attempted to transition to any

sex other than his or her sex,” and the applicant “is willing and able to adhere to all applicable standards, including the standards associated with his or her sex.” *Id.* at 179a-180a.

Those differences, however, are of no constitutional significance. The Department’s prior Carter, Mattis, and Austin policies all recognized that gender dysphoria is a medical condition that poses *some* risk to military readiness. See, e.g., App., *infra*, 4a. That is why all of those policies included gender dysphoria on the military’s list of presumptively disqualifying conditions. See pp. 6-10, *supra*. In narrowing the circumstances in which that presumption of disqualification may be overcome, the 2025 policy simply reflects a more cautious approach to those risks—which is precisely the kind of exercise of professional military judgment that the Constitution does not authorize courts to second-guess. See *Winter*, 555 U.S. at 24 (“[This Court] ‘give[s] great deference to the professional judgment of military authorities concerning the relative importance of a particular military interest.’”) (quoting *Goldman*, 475 U.S. at 507).

b. The 2025 retention standards also satisfy rational-basis review. The 2025 policy requires current servicemembers who have never been diagnosed with gender dysphoria—and who have never received related medical interventions—to serve in their sex and to meet the standards and requirements applicable to their sex. App., *infra*, 126a. As noted, the Carter, Mattis, and Austin policies required the same thing. See pp. 7, 9, 10, *supra*. That requirement of the 2025 policy—and of the Carter, Mattis, and Austin policies before it—rests on the Department’s rational determination that a servicemember who is not experiencing “clinically significant distress or impairment” has no grounds to be excused from serving in the member’s sex or from meeting the standards and requirements applicable to that sex. *DSM-5*, at 453.

Where the 2025 retention standards differ from the Carter and Austin retention standards is in their approach to “gender transition” as a treatment for gender dysphoria. The Carter and Austin retention standards permitted servicemembers who were diagnosed with gender dysphoria to undergo “gender transition” and serve in their asserted gender identity. See pp. 7, 10, *supra*. In contrast, the 2025 retention standards, like the Mattis retention standards, generally disqualify servicemembers who are diagnosed with gender dysphoria instead of allowing them to undergo “gender transition.” See pp. 8-9, 12, *supra*. As above, however, the differences between the policies are constitutionally immaterial. The 2025 policy, like the Mattis policy before it, rests on the Department’s considered military judgment that “making accommodations for gender transition” would “likely undermine” “military effectiveness and lethality.” App., *infra*, 51a; see *id.* at 122a.

First, accommodating gender transition as a treatment for gender dysphoria would “present a significant challenge for unit readiness.” App., *infra*, 45a. The report accompanying the Mattis policy, which the Department considered in formulating the 2025 policy, noted the existence of “considerable scientific uncertainty” concerning whether “transition-related” interventions, such as “cross-sex hormone therapy” and “sex reassignment surgery,” “fully remedy * * * the mental health problems associated with gender dysphoria.” *Id.* at 42a; see *id.* at 121a. The Mattis report reasoned, moreover, that even if such interventions could fully remedy the “serious problems associated with gender dysphoria,” most servicemembers undergoing such interventions could be rendered “non-deployable for a potentially significant amount of time.” *Id.* at 45a. The report noted, for example, that some servicemembers would have to leave their “theater of operations” to be able to undergo “cross-sex hormone therapy or sex reassignment surgery.” *Id.* at 43a. In formulating the 2025 policy, the

Department also considered more recent reviews that reinforced the same points, including a 2025 medical-literature review that reported that “the strength of evidence on transgender mental health and gender-affirming care is low to moderate,” and a 2021 review that “found that nearly 40% of Service members with gender dysphoria in an observed cohort were non-deployable over a 24 month period.” *Id.* at 121a-122a.

Second, accommodating gender transition as a treatment for gender dysphoria would undermine “unit cohesion and good order and discipline.” App., *infra*, 46a; see *id.* at 45a-51a. The military maintains separate berthing, bathroom, and shower facilities for each sex. *Id.* at 45a. The Mattis report expressed concern that allowing individuals who retained the anatomy of their sex to use the facilities of their preferred gender “would invade the expectations of privacy” of the other servicemembers sharing those facilities. *Id.* at 47a; see *United States v. Virginia*, 518 U.S. 515, 550 n.19 (1996) (recognizing that it is “necessary to afford members of each sex privacy from the other sex in living arrangements”). The military also maintains different sets of physical-fitness, body-fat, uniform, and grooming standards for males and females. App., *infra*, 45a. The Mattis report expressed concern, for instance, that allowing a “biological male” to “compete against females in gender-specific physical training” would pose a serious safety risk and generate perceptions of unfairness. *Id.* at 41a; see *id.* at 39a; *Virginia*, 518 U.S. at 550 n.19 (acknowledging that it is “necessary” to “adjust aspects of the physical training programs” for servicemembers to address biological differences between the sexes).

Third, accommodating gender transition as a treatment for gender dysphoria would be “disproportionately costly on a per capita basis.” App., *infra*, 51a. The Mattis report cited the Department’s own experience under the Carter policy. *Ibid.* The report explained that, since implementation of the Carter policy, medical costs

for servicemembers with gender dysphoria had increased nearly 300% compared to servicemembers without gender dysphoria. *Ibid.* Several commanders had also reported that providing servicemembers in their units with transition-related interventions required the use of “operations and maintenance funds to pay for * * * extensive travel throughout the United States to obtain specialized medical care.” *Ibid.* Particularly “in light of the absence of solid scientific support for the efficacy of [transition-related] treatment,” the Mattis report found the costs of accommodating “gender transition” disproportionate. *Ibid.* And in formulating the 2025 policy, the Department also considered a more recent review that found that, “between 2015 and 2024, DoD spent \$52,084,407 providing care to active duty Service members to treat gender dysphoria.” *Id.* at 122a.

In deciding not to accommodate gender transition as a treatment for gender dysphoria, the Department specifically considered—and rejected—the “prior DoD policy” of the past few years that took a contrary approach to gender transition. App., *infra*, 121a. That “studied choice of one alternative in preference to another,” *Rostker*, 453 U.S. at 72, in light of “military operations and needs,” *id.* at 68, is precisely the type of judgment deserving of deference, *ibid.* The Constitution does not authorize courts to second-guess the approach to gender transition adopted by the 2025 policy (and by the Mattis policy before it).

3. Neither court below identified any relevant difference between the 2025 policy and the Mattis policy

In staying the injunctions against the Mattis policy, this Court necessarily determined that the Mattis policy was likely consistent with equal protection. See *Karnoski*, 586 U.S. at 1124; *Stockman*, 586 U.S. at 1124. Neither court below identified

anything about the 2025 policy that would justify a different conclusion as to its constitutionality.

The Ninth Circuit did not even attempt to identify any difference between the two policies, denying a stay solely based on the government’s purported lack of irreparable harm. App., *infra*, 258a. And while the district court stated that the 2025 policy “goes further than” the Mattis policy, it failed to identify any constitutionally relevant difference. *Id.* at 193a. Under the 2025 policy, as under the Mattis policy, “persons who are diagnosed with, or have a history of, gender dysphoria are generally disqualified from accession or retention in the Armed Forces.” *Id.* at 52a; see *id.* at 129a, 131a. The 2025 policy merely narrows the circumstances in which such a person may nevertheless serve. For example, the 2025 policy, unlike the Mattis policy, requires a showing of “a compelling Government interest” for an otherwise disqualified person to obtain a waiver. *Id.* at 179a. And the 2025 policy, unlike the Mattis policy, provides no exemption for current servicemembers who have already received a diagnosis of gender dysphoria. See *id.* at 15a-16a. But those differences between the 2025 policy and the Mattis policy—like any differences between the 2025 policy and the Carter and Austin policies—simply reflect a more cautious approach to the “risks associated with allowing the accession and retention of individuals with a history or diagnosis of gender dysphoria.” *Id.* at 8a; see pp. 21, 24, *supra*. Nothing in the Constitution forbids such an approach.

The district court also suggested that the 2025 policy could not withstand rational-basis review without “updated data”—*i.e.*, data showing that the judgments underlying the Mattis policy are still valid. App., *infra*, 236a. That suggestion is mistaken for several reasons. First, under rational-basis review, the government has no burden to “produce evidence” in support of a policy *at all*; instead, the plaintiff

bears the burden “to negative every conceivable basis which might support” the policy, “whether or not the basis has a foundation in the record.” *Heller v. Doe*, 509 U.S. 312, 320-321 (1993) (citation omitted). Second, the fact that, during the last Administration, the Austin policy took a different approach to the risks associated with allowing the accession and retention of individuals with a history or diagnosis of gender dysphoria does not mean that those risks do not exist—let alone foreclose the 2025 policy from taking a more cautious approach to those risks. In relying on experience under the Austin policy to suggest that those risks are not meaningful, the court improperly second-guessed the military’s own assessment of those risks. And third, in formulating the 2025 policy, the Department *did* consider more recent studies, which reinforced the risks that the Mattis report had identified. App., *infra*, 121a-122a; see D. Ct. Doc. 71-3 (Mar. 12, 2025); D. Ct. Doc. 71-4 (Mar. 12, 2025). The 2025 policy thus readily satisfies rational-basis review.

B. Respondents’ Other Challenges To The 2025 Policy Lack Merit

Respondents also raise First Amendment, due process, and equitable estoppel challenges to the 2025 policy. Those challenges are similar to the First Amendment and due process challenges that this Court considered—and found unlikely to succeed—in granting stays of the injunctions against the Mattis policy. See, *e.g.*, Pet. at 25, *Karnoski, supra* (No. 18-676); Gov’t Stay Appl. at 28, *Karnoski, supra* (No. 18A625). The government is similarly likely to succeed on respondents’ remaining challenges in this case.

1. Respondents contend that the 2025 policy violates the First Amendment because it “prohibits transgender people from disclosing that they are transgender or expressing a gender identity that is different from their sex assigned at birth.” D. Ct. Doc. 23, at 28 (Feb. 19, 2025). But the 2025 policy does not “prohibit[] transgender

people from disclosing that they are transgender.” *Ibid.* Like the *DSM-5* on which it is based, the 2025 policy “focuses on dysphoria as the clinical problem, not identity per se.” *DSM-5*, at 451. Nothing under the 2025 policy turns on a person’s identity—let alone whether a person “disclos[es]” a particular identity. D. Ct. Doc. 23, at 28.

The 2025 policy, like the Carter, Mattis, and Austin policies that preceded it, requires servicemembers who have never been diagnosed with gender dysphoria—and who have never received related interventions—to serve in their sex and to meet the standards and requirements applicable to their sex. App., *infra*, 126a; see pp. 7, 9, 10, *supra*. But that does not violate the First Amendment, any more than it did under the Carter, Mattis, and Austin policies. Requiring servicemembers to use certain berthing, bathroom, and shower facilities is a regulation of conduct, not speech. See, e.g., *Sorrell v. IMS Health Inc.*, 564 U.S. 552, 567 (2011) (distinguishing “restrictions on protected expression” from “restrictions on * * * nonexpressive conduct,” and reaffirming that “the First Amendment does not prevent restrictions directed at * * * conduct from imposing incidental burdens on speech”). And requiring servicemembers to meet certain uniform and grooming standards “foster[s] instinctive obedience, unity, commitment, and esprit de corps” while burdening expression only incidentally, if at all. *Goldman*, 475 U.S. at 507. “The essence of military service ‘is the subordination of the desires and interests of the individual to the needs of the service.’” *Ibid.* (citation omitted). The First Amendment does not require the military to make exception to its uniform and grooming standards to accommodate the personal preferences of particular servicemembers. See *id.* at 508-510.

Respondents also contend that the 2025 policy violates the First Amendment by requiring the use of pronouns that “reflect a Service member’s sex.” App., *infra*, 126a; see D. Ct. Doc. 23, at 28. “But ‘within the military community there is simply

not the same individual autonomy as there is in the larger civilian community.” *Goldman*, 475 U.S. at 507 (brackets and citation omitted). The policy on pronoun use is merely one of many protocols governing how servicemembers should be addressed within the military community. The First Amendment does not stand in the way of such efforts to maintain “good order and discipline” within military ranks. App., *infra*, 126a; see *Goldman*, 475 U.S. at 507 (“The military must insist upon * * * a discipline without counterpart in civilian life’ in order to prepare for and perform its vital role.”) (brackets and citation omitted).

2. Respondents’ contention that the 2025 policy violates procedural due process also fails. Respondents have no constitutionally protected liberty or property interest in continued military service or the employment benefits that come with military service. See *Board of Regents v. Roth*, 408 U.S. 564, 569-579 (1972) (holding that a non-tenured professor at a state university had no constitutionally protected liberty or property interest in continued employment). And to the extent respondents assert that current servicemembers have a constitutionally protected liberty interest in avoiding the “reputational harm” from being discharged under the 2025 policy, respondents are mistaken. D. Ct. Doc. 23, at 31. The 2025 policy provides that the Department will characterize the service of anyone who is discharged as “honorable except where the Service member’s record otherwise warrants a lower characterization.” App., *infra*, 126a. The 2025 policy therefore does not threaten the reputational harm that respondents allege.

In any event, respondents’ claim is not that the Department must follow additional *procedures* before discharging servicemembers under the 2025 policy; it is that servicemembers should not be discharged under the 2025 policy *at all*. See D. Ct. Doc. 23, at 32-33. Indeed, respondents acknowledge that they “do not challenge the

procedures that will be used to determine whether, for example, a particular individual has a history of gender dysphoria.” D. Ct. Doc. 82, at 22 (Mar. 19, 2025). Instead, respondents “challenge the *basis*” for their “disqualification as unconstitutional.” *Ibid.* (emphasis added). That is a *substantive* due process challenge, not a *procedural* one. See *Connecticut Dep’t of Pub. Safety v. Doe*, 538 U.S. 1, 8 (2003) (finding a similar claim to be “actually a substantive challenge * * * ‘recast in “procedural due process” terms’”) (quoting *Reno v. Flores*, 507 U.S. 292, 308 (1993)). And because the 2025 policy is rationally related to legitimate government interests, respondents’ due process challenge fails. See pp. 19-26, *supra*.

3. Finally, respondents assert that the Department is equitably estopped from implementing the 2025 policy against current servicemembers who have undergone gender transition while serving in the military. D. Ct. Doc. 23, at 33-35. But this Court has “reversed every finding of estoppel” against the government that it has reviewed, and there is no basis for estoppel here. *OPM v. Richmond*, 496 U.S. 414, 422 (1990). To begin, estoppel may never run against the government. See *id.* at 423 (reserving the question “whether an estoppel claim could ever succeed against the Government”). But even if it could, it would require a showing of “some type of ‘affirmative misconduct.’” *Id.* at 421 (citation omitted). Respondents make no such showing here. They allege that the Department adopted the 2025 policy after permitting current servicemembers to undergo gender transition under previous policies. D. Ct. Doc. 23, at 34. But a mere policy change—here, one based on a reevaluation of the risks associated with accommodating gender transition, see App., *infra*, 122a—cannot be described as misconduct, let alone affirmative misconduct.

C. At Minimum, The District Court Erred In Granting A Universal Injunction

Even setting aside the district court’s errors on the merits, the court improperly enjoined the 2025 policy on a universal basis. The court should have limited any injunction to the eight individual respondents in this case. *App., infra*, 140a-141a.

1. Nationwide or universal remedies exceed “the power of Article III courts,” conflict with “longstanding limits on equitable relief,” and impose a severe “toll on the federal court system.” *Hawaii*, 585 U.S. at 713 (Thomas, J., concurring); see *Department of State v. AIDS Vaccine Advocacy Coal.*, 145 S. Ct. 753, 756 (2025) (Alito, J., dissenting); *Labrador v. Poe*, 144 S. Ct. 921, 923-924 (2024) (Gorsuch, J. concurring); *DHS v. New York*, 140 S. Ct. 599, 599-601 (2020) (Gorsuch, J., concurring).

Start with the constitutional problem: Article III authorizes federal courts to exercise only “judicial Power,” which extends only to “Cases” and “Controversies.” U.S. Const. Art. III, § 2, Cl. 1. Under that power, courts can adjudicate “claims of infringement of individual rights,” “whether by [the] unlawful action of private persons or by the exertion of unauthorized administrative power.” *Lujan v. Defenders of Wildlife*, 504 U.S. 555, 577 (1992) (citation omitted). Courts that sustain such claims may grant the challenger appropriate relief—for instance, an injunction preventing the enforcement of a challenged law or policy against that individual—but cannot grant relief to strangers to the litigation. Article III does not empower federal courts to “exercise general legal oversight of the Legislative and Executive Branches.” *TransUnion LLC v. Ramirez*, 594 U.S. 413, 423-424 (2021). To reach beyond the litigants and to enjoin the Executive Branch’s actions toward third parties “would be not to decide a judicial controversy, but to assume a position of authority over the

governmental acts of another and co-equal department, an authority which plainly [courts] do not possess.” *Massachusetts v. Mellon*, 262 U.S. 447, 489 (1923).

Universal injunctions also contravene this Court’s precedents on Article III standing. “[S]tanding is not dispensed in gross,” so plaintiffs must establish standing “for each form of relief that they seek.” *Murthy v. Missouri*, 603 U.S. 43, 61 (2024) (citations omitted). And a plaintiff’s remedy must be “limited to the inadequacy that produced his injury in fact.” *Gill v. Whitford*, 585 U.S. 48, 66 (2018) (brackets and citation omitted). Even if respondents have standing to seek relief for themselves, they lack standing to seek relief for third parties, as to whom plaintiffs cannot “sufficiently answer the question: ‘What’s it to you?’” *TransUnion*, 594 U.S. at 423 (citation and internal quotation marks omitted).

Universal injunctions also transgress restrictions on courts’ equitable powers. Federal courts sitting in equity must apply “traditional principles of equity jurisdiction” and may award only those remedies that were “traditionally accorded by courts of equity.” *Grupo Mexicano de Desarrollo S.A. v. Alliance Bond Fund, Inc.*, 527 U.S. 308, 319 (1999) (citation omitted). Congress may by statute authorize new remedies, but courts may not on their own authority “create remedies previously unknown to equity jurisprudence.” *Id.* at 332; see *Alexander v. Sandoval*, 532 U.S. 275, 286 (2001) (new remedies “must be created by Congress”).

American courts of equity traditionally “did not provide relief beyond the parties to the case.” *Hawaii*, 585 U.S. at 717 (Thomas, J., concurring). They have instead long followed the “rule that injunctive relief should be no more burdensome to the defendant than necessary to provide complete relief to the plaintiffs.” *Califano v. Yamasaki*, 442 U.S. 682, 702 (1979). Unsurprisingly, then, there appear to have been “no national injunctions against federal defendants for the first century and a half of

the United States.” Samuel L. Bray, *Multiple Chancellors: Reforming the National Injunction*, 131 Harv. L. Rev. 417, 428 (2017).

Instead, in a 19th-century case where a lower court issued a universal injunction against the enforcement of a *state* statute, this Court agreed that the challenged statute violated the Constitution, see *Scott v. Donald*, 165 U.S. 58, 99-101 (1897), but nonetheless held in a separate opinion that the universal injunction was unlawful and that relief should have been “restricted to the part[y] named as plaintiff,” *Scott v. Donald*, 165 U.S. 107, 117 (1897). And in a similar modern-day precedent, this Court agreed that a statute prohibiting federal employees from accepting honoraria violated the First Amendment, but held that the injunction protecting “any Executive Branch employee” was overbroad and had to be “limited to the parties before the Court.” *United States v. National Treasury Employees Union*, 513 U.S. 454, 462, 477 (1995). The Court considered it inappropriate “to provide relief to nonparties when a narrower remedy will fully protect the litigants.” *Id.* at 478.

Universal injunctions also subvert the Article III hierarchy of judicial review. Ordinarily, the coercive effect of a court’s judgment extends only to the case at hand, but the *stare decisis* effect of the court’s opinion may extend to other cases, depending on the court’s position in the Article III hierarchy. A district court’s opinion has no binding precedential effect at all, even in the same district or on the same judge in a different case. See *Camreta v. Greene*, 563 U.S. 692, 709 n.7 (2011). A court of appeals’ published opinion, in turn, constitutes controlling precedent throughout the relevant circuit, though not in other circuits. See *Poe*, 144 S. Ct. at 932 (Kavanaugh, J., concurring). And, of course, this Court’s decisions constitute controlling precedent throughout the Nation. If this Court were to hold a challenged statute or policy unconstitutional, the government could not “successfully enforce [it] against anyone,

party or not, in light of *stare decisis*.” *Griffin v. HM Florida-ORL, LLC*, 144 S. Ct. 1, 1 (2023) (statement of Kavanaugh, J.). When district courts grant universal injunctions, they upend that system, imbuing the orders of courts of first instance with the type of nationwide effect usually reserved for the precedents of the court of last resort.

Further, universal injunctions “render meaningless rules about joinder and class actions.” *United States v. Texas*, 599 U.S. 670, 703 (2023) (Gorsuch, J., concurring in the judgment). They can “sweep up nonparties who may not wish to receive the benefit of the court’s decision.” *Ibid.*; see *Arizona v. Biden*, 40 F.4th 375, 396 (6th Cir. 2022) (Sutton, C.J., concurring) (“Nationwide injunctions * * * sometimes give States victories they do not want.”).

Universal injunctions cause significant harm to the government. They invite forum shopping because different challengers need not file different challenges in different courts if one challenger who files one suit in one court can secure victory nationwide. See *Poe*, 144 S. Ct. at 927 (Gorsuch, J., concurring). They force the government “to seek immediate relief from one court and then the next, with the finish line in this Court.” *Ibid.* They countermand the principle that the government is not subject to non-mutual issue preclusion—*i.e.*, that the government may relitigate an issue against one party even if it has lost that issue against another party in another case. See *United States v. Mendoza*, 464 U.S. 154, 162-163 (1984). And they operate asymmetrically, granting relief to strangers everywhere whenever a single plaintiff prevails, but not precluding continued litigation by others if some plaintiffs lose. See *DHS*, 140 S. Ct. at 601 (Gorsuch, J., concurring).

Finally, universal injunctions harm the courts. “By their nature, universal injunctions tend to force judges into making rushed, high-stakes, low-information decisions.” *DHS*, 140 S. Ct. at 600 (Gorsuch, J., concurring). They exert substantial

pressure on this Court’s emergency docket, forcing the Court to confront difficult issues without “the airing of competing views” among “multiple judges and multiple circuits.” *Ibid.* And they needlessly encourage “[r]epeated and essentially head-on confrontations between the life-tenured branch and the representative branches.” *Valley Forge Christian Coll. v. Americans United for Separation of Church & State, Inc.*, 454 U.S. 464, 474 (1982) (citation omitted).

2. In entering a universal injunction in this case, the district court did not grapple with those principles. Instead, the court issued a “nationwide” injunction merely because of the “nationwide” scope of the 2025 policy. App., *infra*, 255a. But Article III and principles of equity require courts to tailor injunctions to the scope of the plaintiff’s injury, not to the scope of the defendant’s policy. The court’s contrary view “lacks a limiting principle and would make nationwide injunctions the rule rather than the exception with respect to all actions of federal agencies.” *Arizona*, 40 F.4th at 397 (Sutton, C.J., concurring).

Indeed, this Court has previously stayed a nationwide injunction against a military policy to the extent it swept beyond the parties to the case. *United States Dep’t of Def. v. Meinhold*, 510 U.S. 939 (1993). *Meinhold* involved a facial constitutional challenge by a discharged member of the Navy to the Department’s then-existing policy disqualifying servicemembers based on sexual orientation. *Meinhold v. United States Dep’t of Def.*, 34 F.3d 1469, 1472-1473 (9th Cir. 1994). After the district court enjoined the Department from “taking any actions against gay or lesbian servicemembers based on their sexual orientation” nationwide, *ibid.*, this Court stayed that order to the extent it “grant[ed] relief to persons other than [the plaintiff],” *Meinhold*, 510 U.S. at 939.

Any injunctive relief in this case likewise should not extend beyond the eight individual respondents in this case. App., *infra*, 140a-141a. An injunction so limited would fully redress any purported injuries not only to the individual respondents themselves, but also to Gender Justice League (GJL), the organizational respondent in this case. *Id.* at 141a. The only GJL members whom GJL has identified as potentially harmed by the 2025 policy are three of the individual respondents. *Id.* at 118a.

II. THE OTHER FACTORS SUPPORT STAYING THE DISTRICT COURT’S INJUNCTION

In staying the injunctions against the Mattis policy, this Court necessarily determined that the other stay factors—*i.e.*, whether the underlying issues warrant review, whether the government likely faces irreparable harm, and the balance of equities—supported relief. Those factors likewise support relief here.

A. The Questions Presented By The District Court’s Injunction Plainly Warrant This Court’s Review

If the Ninth Circuit were to uphold the district court’s injunction, certiorari would clearly be warranted. Respondents challenge the 2025 policy on equal protection, First Amendment, due process, and equitable estoppel grounds. Those challenges concern a matter of imperative public importance: the authority of the U.S. military to determine who may serve in the Nation’s armed forces. Relying on the conclusions of a panel of experts and other studies and reviews of service by individuals with gender dysphoria, the Department determined that “the medical, surgical, and mental health constraints” on individuals with gender dysphoria “are incompatible with the high mental and physical standards necessary for military service.” App., *infra*, 124a; see *id.* at 121a-122a.

The district court’s injunction nullifies that exercise of professional military judgment and blocks the implementation of a policy that the Department has deemed

necessary to maintain its “rigorous standards” and “deliver a ready, deployable force.” App., *infra*, 122a. If the Ninth Circuit were to affirm the injunction, a judicial intrusion of that significance into the operation of our Nation’s armed forces would unquestionably warrant this Court’s review. See *Department of the Navy v. Egan*, 484 U.S. 518, 520 (1988) (granting certiorari to address interference with Executive Branch determinations that were of “importance * * * to national security concerns”); see also *Winter*, 555 U.S. at 12. Indeed, this Court frequently grants plenary review in response to lower-court decisions blocking significant federal policies or programs, even outside the military context. See, e.g., *Becerra v. Braidwood Mgmt., Inc.*, No. 24-316, 2025 WL 65913 (U.S. Jan. 10, 2025); *Garland v. VanDerStok*, 144 S. Ct. 1390 (2023).

B. The District Court’s Injunction Causes Irreparable Harm To The Government And To The Public

The district court’s injunction irreparably harms the Executive Branch by preventing a branch of government from carrying out its work—here, the Department of Defense’s responsibility for military readiness. The President holds “the mandate of the people to exercise his executive power.” *Myers v. United States*, 272 U.S. 52, 123 (1926). The Executive Branch exists to carry out his policies. While courts may adjudicate the lawfulness of those policies in justiciable cases, they irreparably injure our democratic system when they forbid the government to effectuate those policies against anyone anywhere in the Nation, and all the more so when the policy is lawful. See *Doe #1 v. Trump*, 957 F.3d 1050, 1084 (9th Cir. 2020) (Bress, J., dissenting); cf. *Maryland v. King*, 567 U.S. 1301, 1303 (2012) (Roberts, C.J., in chambers) (“Any time a State is enjoined by a court from effectuating statutes enacted by representatives of its people, it suffers a form of irreparable injury.”) (brackets and citation omitted).

That injury to our democratic system is particularly pronounced in the area of military affairs. The Constitution entrusts to the political branches the authority to determine the composition of the armed forces. See *Winter*, 555 U.S. at 24; *Rostker*, 453 U.S. at 66. The Department exercised that authority in adopting the 2025 policy to exclude “individuals who have a current diagnosis or history of, or exhibit symptoms consistent with, gender dysphoria.” App., *infra*, 124a. The district court’s injunction nevertheless forces the military to maintain a different policy—one that the Department has concluded is inconsistent with “the best interests of the Military Services” and with “the interests of national security.” *Id.* at 126a. The court’s injunction thus irreparably interferes with “the professional judgment of military authorities” in “carrying out our Nation’s military policy.” *Goldman*, 475 U.S. at 507-508.

In denying the government’s motion for a stay, the Ninth Circuit merely asserted, without further analysis, that the government had not demonstrated that it “will suffer irreparable harm absent a stay.” App., *infra*, 258a. But that conclusion cannot be squared with this Court’s decisions to stay the injunctions against the Mattis policy. See *Karnoski*, 586 U.S. at 1124; *Stockman*, 586 U.S. at 1124. In granting those stays, this Court necessarily determined that the government was likely to suffer irreparable harm absent a stay. See *Hollingsworth*, 558 U.S. at 190 (“To obtain a stay * * *, an applicant must show * * * a likelihood that irreparable harm will result from the denial of a stay.”). Just as the injunctions against the Mattis policy irreparably harmed the government by requiring the military to maintain a prior policy that was “not conducive to military effectiveness and lethality,” App., *infra*, 8a, the injunction here irreparably harms the government by requiring the military to do the same. Indeed, if the separation of powers means anything, the government obvi-

ously suffers irreparable harm when an unelected judge usurps the role of the political branches in operating the Nation’s armed forces.

C. The Balance Of Equities Favors Staying The District Court’s Injunction

On the other side of the balance, vacating the district court’s injunction would not cause respondents irreparable harm. Alleging the same types of harms raised by the plaintiffs who unsuccessfully challenged the Mattis policy, the individual respondents assert that the 2025 policy will either lead to their discharge or stand in the way of joining the military in the first place. D. Ct. Doc. 23, at 35-36; see, *e.g.*, Joint Opp. at 30-31, *Karnoski*, *supra* (No. 18A625). But the loss of employment and associated benefits are not *irreparable* harms. See *Sampson v. Murray*, 415 U.S. 61, 92 n.68 (1974) (holding that “an employee’s discharge, together with the resultant effect on the employee,” ordinarily does not amount to irreparable injury). And any assertion that the 2025 policy will cause respondents reputational harm is unfounded, given that any discharge under the policy will be “honorable except where the Service member’s record otherwise warrants a lower characterization.” App., *infra*, 126a. In any event, any harm to respondents is substantially outweighed by the harm to the government and to the public from forcing the military to maintain a policy that the Department has deemed inconsistent with “the best interests of the Military Services” and with “the interests of national security.” *Ibid.*; see *Winter*, 555 U.S. at 23 (holding that “[e]ven if plaintiffs ha[d] shown irreparable injury from the Navy’s training exercises, any such injury [wa]s outweighed by the public interest and the Navy’s interest in effective, realistic training of its sailors”). The balance of equities therefore tips decisively in favor of staying the injunction—just as it did when this Court stayed the injunctions against the Mattis policy.

CONCLUSION

This Court should stay the district court's injunction in its entirety. At minimum, this Court should stay the injunction except as to the eight individual respondents in this case.

Respectfully submitted.

D. JOHN SAUER
Solicitor General

APRIL 2025

APPENDIX

DoD, Directive-Type Memorandum 16-005, <i>Military Service of Transgender Service Members</i> (June 30, 2016) (D. Ct. Doc. 33-1)	1a
DoD, Mattis Memorandum and Report (Feb. 2018) (D. Ct. Doc. 71-2)	7a
DoD Instruction 6130.03, Vol. 1, <i>Medical Standards for Military Service: Appointment, Enlistment, or Induction</i> (May 28, 2024) (D. Ct. Doc. 76-3)	55a
Exec. Order No. 14,183 (Jan. 27, 2025), 90 Fed. Reg. 8757 (Feb. 3, 2025).....	114a
Declaration of Danielle Askini Aubain (Feb. 12, 2025) (D. Ct. Doc. 30)	117a
DoD, <i>Action Memo: Implementing Guidance for Prioritizing Military Excellence and Readiness Executive Order</i> (Feb. 26, 2025) (D. Ct. Doc. 71-1).....	119a
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DoD, <i>Clarifying Guidance on Prioritizing Military Excellence and Readiness: Retention and Accession Waivers</i> (Mar. 4, 2025) (D. Ct. Doc. 64-1)	179a
DoD, <i>Prioritizing Military Excellence and Readiness: Military Department Identification</i> (Mar. 21, 2025) (D. Ct. Doc. 92-1).....	181a
Transcript of preliminary injunction hearing (Mar. 25, 2025)	186a
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